FUTURES MATTER EDUCATION

Futures Matter - Northwood, 122 Emily Street, Highgate, Birmingham, B12 0XJ

Supporting Pupils with Medical Conditions Policy

Reference: FME-SE-02

Last review: September 2026
Next review: September 2027

Reviewed by: Benjamin Neasom, Head Teacher
Carrie-Ann McGurk, Proprietor
Mark O'Donnell, Chair of School Governance Board

CONTENTS

1.0AIMS 3
2.0LEGISLATION AND STATUTORY RESPONSIBILITIES 3
3.0ROLES AND RESPONSIBILITIES 3
4.0EQUAL OPPORTUNITIES 5
5.0BEING NOTIFIED THAT A PUPIL HAS A MEDICAL CONDITION 5
6.0INDIVIDUAL HEALTHCARE PLANS 6
7.0MANAGING MEDICINES 7
7.1Controlled drugs 7
7.2Pupils managing their own needs 7
7.3Unacceptable practice 8
8.0EMERGENCY PROCEDURES 8
9.0TRAINING 8
10.0RECORD KEEPING 9
11.0LIABILITY AND INDEMNITY 9
12.0COMPLAINTS AND MONITORING 10
13.0LINKS WITH OTHER POLICIES 10
14.0APPENDICES 10
APPENDIX 1: BEING NOTIFIED OF A MEDICAL CONDITION 11
APPENDIX 2: SICK OR INFECTIOUS PUPILS 12
APPENDIX 3: INDIVIDUAL HEALTHCARE PLAN 14
APPENDIX 4: PARENTAL AGREEMENT FOR MEDICINE 16

APPENDIX 5: RECORD OF MEDICINE GIVEN 17

1.0 AIMS

At Futures Matter - Northwood we understand that medical conditions that need support at school can affect quality of life and may be life-threatening. Many of our pupils also have social, emotional and mental health needs and autism, and a number live with a long-term physical or mental health condition as well. We will support pupils with medical conditions so that they have full access to education, including school trips and physical education.

This policy aims to:

- make sure that pupils, staff and parents or carers understand how we will support pupils with medical conditions
- set out the roles and responsibilities of everyone in the school community in regard to pupils with medical conditions
- set out the procedure for creating, reviewing and managing individual healthcare plans (IHPs)
- set out how we manage medicines in school
- reassure parents and carers that the school will help their child feel safe, supported and included

The person with overall responsibility for putting this policy into practice is the Head Teacher, Benjamin Neasom. He can be contacted on 0121 835 1360 or at benjamin.neasom@futuresmattereducation.co.uk.

2.0 LEGISLATION AND STATUTORY RESPONSIBILITIES

This policy meets the requirements of the Education (Independent School Standards) Regulations 2014, which require us to have and follow a policy on the welfare, health and safety of pupils, including the arrangements for pupils who are ill. It is based on the DfE statutory guidance Supporting pupils at school with medical conditions. Section 100 of the Children and Families Act 2014 places a duty on the governing bodies of maintained schools and academies to make arrangements to support pupils with medical conditions. Independent schools are not bound by that duty in the same way, but we follow the guidance as good practice and as part of our commitment to our pupils.

It also takes account of the Equality Act 2010, the Misuse of Drugs Regulations 2001, the DfE guidance Arranging education for children who cannot attend school because of health needs, and the UK Health Security Agency (UKHSA) guidance Health protection in education and childcare settings.

3.0 ROLES AND RESPONSIBILITIES

Supporting pupils with medical conditions during school hours is not the sole responsibility of one person. The people below have particular responsibilities.

Who	Responsibilities
The Proprietor and the School Governance Board	Have ultimate responsibility for making sure there are arrangements to support pupils with medical conditions. Determine and approve this policy, review it on time, and monitor practice and staff training in line with it. Make sure the school holds the right insurance (section 11). The Chair of the Board, Mark O'Donnell, checks with the Head Teacher each term that IHPs are in place for the pupils who need them.
The Head Teacher	Is responsible for putting this policy into practice and leads on IHPs, because the school does not have a separate SENCo. Makes sure all staff know about this policy and understand their role. Makes sure enough trained staff are available to deliver all IHPs, including in contingency and emergency situations. Makes sure that staff who need to know are aware of a pupil's condition. Makes sure staff are appropriately insured and know that they are. Manages cover for staff absence or turnover, so a suitable member of staff is always available, and briefs supply staff on pupils' medical needs. Approves risk assessments for school visits and activities outside the normal school timetable that involve pupils with medical conditions. Contacts the school nursing service about any pupil with a medical condition who may need support at school but has not yet been brought to the nurse's attention. Makes sure there are systems for getting information about a pupil's medical needs and keeping it up to date.
The Designated Safeguarding Lead (DSL)	Considers whether a pupil's medical needs, or a pattern of illness or missed medication, raise a safeguarding concern, such as neglect. The Head Teacher is the DSL and the Deputy DSLs are the Proprietor, Carrie-Ann McGurk, and Simone Welch.
The first aider and medication lead	Kiana Gibson-Sawyers holds a 3 day first aid qualification and is the member of staff who gives medicine to pupils, following each pupil's IHP and the Administering Medicines Policy (FME-HS-13). She keeps the record of medicine given (Appendix 5), checks that medicines are in date and correctly labelled, and tells the Head Teacher about any error or near miss straight away. When she is not in school, the Head Teacher names a suitably trained member of staff to cover.
Staff	Any member of staff may be asked to provide support to pupils with medical conditions, including giving medicines, although they cannot be required to do so. Staff who take on this responsibility receive suitable training and reach the necessary level of competence before they start. Teachers take into account the needs of pupils with medical conditions in their lessons. All staff know what to do and respond when they become aware that a pupil with a medical condition needs help.

Who	Responsibilities
Parents and carers	Give the school enough up-to-date information about their child's medical needs. Provide evidence of the prescription and written permission for medicines to be given by staff. Are involved in developing and reviewing their child's IHP. Carry out any action they have agreed to, for example supplying medicines and equipment and making sure they or another named adult can be reached at all times.
Pupils	Are often best placed to tell us how their condition affects them. They are fully involved in discussions about their medical support needs and help to develop their IHPs, and are expected to follow them.
School nurses and other healthcare professionals	The school nursing service is asked to tell the school when a pupil has been identified as having a medical condition that will need support in school, before the pupil starts where possible, and may support staff to put an IHP into practice. GPs, paediatricians, CAMHS and specialist nurses are asked to give advice on developing IHPs and to confirm that staff are competent to carry out any medical procedure.

4.0 EQUAL OPPORTUNITIES

We follow our legal responsibilities under the Equality Act 2010 and will not unlawfully discriminate against any pupil. We are clear about the need to actively support pupils with medical conditions to take part in school trips and visits and in sporting activities, and not to prevent them from doing so.

We will consider what reasonable adjustments are needed to enable these pupils to take part fully and safely. Risk assessments will take account of any steps needed to include pupils with medical conditions, and we will consult the pupil, their parents or carers and any relevant healthcare professionals when we plan. Where a medical condition affects a pupil's learning, we will also consider it alongside their EHC plan and the support set out in our SEND Policy.

5.0 BEING NOTIFIED THAT A PUPIL HAS A MEDICAL CONDITION

How we find out. We ask about medical needs at every stage. The admissions process and the Fresh Start conversation before a pupil joins us ask about conditions, medicines, allergies and any healthcare professionals involved. The EHC plan and any information from the previous school or the placing local authority are read for medical information. We ask every family to complete and return a medical information form at the start of each school year, and we ask parents and carers to tell us straight away, by phoning 0121 835 1360 or emailing office@futuresmattereducation.co.uk, if their child's needs change during the year.

What happens next. When the school is told that a pupil has a medical condition, we follow the process in Appendix 1 to decide whether the pupil needs an IHP. We make every effort to put arrangements in place within 2 weeks, or by the start of the relevant term for pupils who are new to the school. Where a pupil has a serious condition, we do not wait for the

plan to be finished before we put sensible interim arrangements in place with the family, and we make sure the staff working with the pupil know what to do.

6.0 INDIVIDUAL HEALTHCARE PLANS

The Head Teacher has overall responsibility for developing IHPs for pupils with medical conditions. Plans are reviewed at least once a year, or earlier if there is evidence that the pupil's needs have changed, for example after a hospital stay or a change of medication.

Plans are developed with the pupil's best interests in mind and set out what needs to be done, when, and by whom. Not every pupil with a medical condition will need an IHP. We will agree with a healthcare professional and the parents or carers when an IHP would be inappropriate or disproportionate, based on evidence. If we cannot agree, the Head Teacher makes the final decision.

Plans are drawn up with the pupil (wherever appropriate), the parents or carers, and a relevant healthcare professional, such as the school nurse, a specialist or a paediatrician, who can best advise on the pupil's needs. An IHP is linked to, or becomes part of, the pupil's EHC plan and Individual Support Plan. It is read alongside any Positive Behaviour Support Plan, because pain, medication side effects, tiredness and anxiety can all affect behaviour. Where a pupil's condition needs a specific risk assessment, this is carried out under our Health and Safety Policy.

The level of detail depends on how complex the condition is and how much support is needed. The Head Teacher will consider the following when deciding what to record on an IHP:

- the medical condition, its triggers, signs, symptoms and treatments
- the pupil's resulting needs, including medication (dose, side effects and storage) and other treatments, time, facilities, equipment, testing, access to food and drink where this is used to manage the condition, dietary requirements, and environmental issues, for example crowded corridors or travel time between lessons
- specific support for the pupil's educational, social and emotional needs, for example how absences will be managed, extra time in exams, rest periods, additional help to catch up with lessons, and counselling
- the level of support needed, including in an emergency. If a pupil manages their own medication, this is clearly stated with arrangements for monitoring
- who will provide the support, their training needs, the expectations of their role, confirmation from a healthcare professional that they are competent, and cover for when they are not available
- who in the school needs to know about the pupil's condition and the support required
- written permission from parents or carers and the Head Teacher for medication to be given by a member of staff, or taken by the pupil themselves, during school hours
- separate arrangements for school trips or other activities outside the normal timetable, so the pupil can take part, for example risk assessments
- where the parent, carer or pupil has asked for confidentiality, the people who will be trusted with information about the pupil's condition
- what to do in an emergency, including who to contact, and contingency arrangements

A template for an IHP is in Appendix 3. IHPs are kept securely on the school's management information system, with a printed emergency summary in a place that all staff who need it know about, and copies are shared with the family.

7.0 MANAGING MEDICINES

We will only give prescription and non-prescription medicines at school when it would be detrimental to the pupil's health or school attendance not to do so, and when we have the written consent of the parent or carer (Appendix 4). Medicine is given by Kiana Gibson-Sawyers, our first aid trained member of staff, or by the trained member of staff named to cover her. The person giving the medicine keeps a written record (Appendix 5). We tell the parent or carer on the same day that the medicine has been given, or as soon as reasonably possible. The only exception is where the medicine has been prescribed to the pupil without their parents' or carers' knowledge.

Pupils under 16 will not be given medicine containing aspirin unless it has been prescribed by a doctor. Anyone giving a pupil medication, for example for pain relief, will first check the recommended and maximum dosages for the pupil's age and when the previous dose was taken. Our staff do not give any medicine that has not been supplied by the parent or carer under this policy.

We will only accept prescribed medicines that are:

- in date
- labelled
- provided in the original container, as dispensed by the pharmacist, with instructions for giving, dosage and storage

We accept insulin that is inside an insulin pen or pump rather than its original container, but it must be in date.

All medicines are stored safely. Pupils are told where their medicines are at all times and can get to them immediately. Medicines and devices such as asthma inhalers, blood glucose testing meters and adrenaline pens are always readily available to pupils and are not locked away. Pupils with prescribed adrenaline devices carry two, or they are kept unlocked in the medical room. The school also holds spare adrenaline auto-injectors in pairs, which may be used in an emergency for any pupil without prior consent, as set out in the Allergy Safety Policy (FME-HS-12). Medicines that need to be kept cool are stored in a fridge that pupils cannot freely get into. When medicines are no longer needed, or are out of date, they are returned to parents or carers to arrange safe disposal.

7.1 Controlled drugs

Controlled drugs are prescription medicines controlled under the Misuse of Drugs Regulations 2001 and later amendments, such as morphine or methadone. A pupil who has been prescribed a controlled drug may keep it in their possession if they are competent to do so, but they must not pass it to another pupil. All other controlled drugs are kept in a secure cupboard in the school office and only named staff have access. They are easily accessible in an emergency, and we keep a record of any doses used and the amount held.

7.2 Pupils managing their own needs

Pupils who are competent are encouraged to take responsibility for managing their own medicines and procedures. We discuss this with parents and carers and record it in the IHP.

Pupils are allowed to carry their own medicines and relevant devices wherever possible. IHPs include a procedure for staff to follow if a pupil refuses to carry out a necessary procedure or to take their medicine. Because some of our pupils find routines around medication hard, we agree these steps with the pupil in advance and keep them calm and private.

7.3 Unacceptable practice

Staff use their discretion and judge each case on its merits with reference to the pupil's IHP, but they keep in mind that it is not generally acceptable practice to:

- prevent pupils from easily getting to their inhalers and medication, or from taking their medication when and where necessary
- assume that every pupil with the same condition needs the same treatment
- ignore the views of the pupil or their parents or carers
- ignore medical evidence or opinion
- send children with medical conditions home frequently for reasons associated with their condition, or stop them from staying for normal school activities, including lunch, unless this is specified in their IHPs
- send an ill pupil to the school office or medical room unaccompanied or with someone unsuitable, for example a fellow pupil who is not old or responsible enough
- penalise pupils for their attendance record if their absences are related to their medical condition, for example hospital appointments
- prevent pupils from drinking, eating or taking toilet or other breaks whenever they need to in order to manage their condition effectively
- require parents or carers, or otherwise make them feel obliged, to come to school to give medication or provide medical support to their child, including with toileting issues. No parent or carer should have to give up working because the school is failing to support their child's medical needs
- prevent pupils from taking part, or create unnecessary barriers to taking part, in any aspect of school life, including school trips, for example by requiring parents or carers to accompany their child
- give medicine, or ask pupils to give medicine, in school toilets

8.0 EMERGENCY PROCEDURES

Staff follow the school's normal emergency procedures, for example calling 999. Every IHP clearly sets out what counts as an emergency for that pupil and explains what to do. If a pupil needs to go to hospital, staff stay with the pupil until the parent or carer arrives, or go with the pupil to hospital in the ambulance. We never take a pupil to hospital in a staff member's car if an ambulance has been called, and we take the pupil's IHP, medicines and emergency contact details with us. The Head Teacher, or in his absence the Deputy DSL, tells the pupil's parents or carers and, where appropriate, the placing local authority and social worker as soon as it is safe to do so. The school's First Aid Policy sets out first aid arrangements and the recording of accidents.

9.0 TRAINING

Staff who are responsible for supporting pupils with medical needs receive suitable and sufficient training. Training needs are identified when IHPs are developed or reviewed, and

staff who will support a pupil are included in the meetings where this is discussed. The relevant healthcare professionals lead on identifying the type and level of training required and agree this with the Head Teacher. Training is kept up to date.

Training will:

- be enough to make sure that staff are competent and confident in their ability to support the pupils
- meet the requirements in the IHPs
- help staff to understand the specific medical conditions they are asked to deal with, their implications and preventative measures

Healthcare professionals confirm the competence of staff in any medical procedure or in giving medication. All staff receive training so that they know about this policy and understand their role in putting it into practice, for example in preventative and emergency measures so that they can recognise a problem and act quickly. This is part of induction for new staff and is refreshed each year.

10.0 RECORD KEEPING

The Head Teacher makes sure that written records are kept of all medicine given to pupils for as long as these pupils are at the school. Parents and carers are told if their child has been unwell at school. IHPs are kept in a readily accessible place that all staff who need them know about. All records are held securely and handled in line with the UK GDPR and the Data Protection Act 2018, and shared only with people who need the information to keep the pupil safe.

11.0 LIABILITY AND INDEMNITY

The Proprietor makes sure that the school holds an appropriate level of insurance that reflects the school's level of risk. This covers staff who support pupils with medical conditions, including the giving of medication. Where a pupil needs a specialist healthcare procedure, the Head Teacher checks with the insurer, before staff are trained, that the cover applies, and arranges any extra cover that is needed.

The school's insurance is held with Probitas 1492 Services Limited under policy number BA299V26A012137, for the period 1 September 2026 to 31 August 2027, and is arranged through Arthur J. Gallagher Insurance Brokers Limited. The cover in place is:

Type of cover	Indemnity limit and excess
Employers Liability	£10,000,000 any one occurrence
Public Liability	£10,000,000 any one occurrence (excess £250)
Products Liability	£10,000,000 in the aggregate (excess £250)
Abuse Liability	£5,000,000 in the aggregate (excess £250)
Treatment Liability	£5,000,000 in the aggregate (excess £1,500)
Professional Indemnity	£2,000,000 in the aggregate (excess £1,500)

To be confirmed in writing with the broker (Peter Jones, Arthur J. Gallagher, 0161 529 1502) by Carrie-Ann McGurk, Proprietor: that the Treatment Liability cover applies to staff giving medicine to pupils, including Kiana Gibson-Sawyers, and to any specialist healthcare procedure listed in a pupil's IHP. Delete this note and the highlighting once it is confirmed.

12.0 COMPLAINTS AND MONITORING

Parents and carers with a complaint about the school's actions in regard to their child's medical condition should first discuss it directly with the Head Teacher. If the Head Teacher cannot resolve the matter, he will direct them to the school's Complaints Policy.

This policy is monitored by the Head Teacher, who reports to the School Governance Board each term on the number of pupils with IHPs, whether they are up to date, medicines errors and near misses, and staff training. It is reviewed and approved by the Proprietor and the School Governance Board every year.

13.0 LINKS WITH OTHER POLICIES

- First Aid Policy
- Administering Medicines Policy (FME-HS-13) and Allergy Safety Policy (FME-HS-12)
- Health and Safety Policy
- Safeguarding and Child Protection Policy
- SEND Policy
- Attendance Policy
- Accessibility Plan
- Equality Information and Objectives
- Complaints Policy

References: Supporting pupils at school with medical conditions, DfE; Arranging education for children who cannot attend school because of health needs, DfE; Health protection in education and childcare settings, UKHSA; Section 100 of the Children and Families Act 2014; The Equality Act 2010; Model policy: supporting pupils with medical conditions (trusts), The Key (October 2025).

14.0 APPENDICES

Appendix 1: Being Notified of a Medical Condition

Appendix 2: Sick or Infectious Pupils

Appendix 3: Individual Healthcare Plan

Appendix 4: Parental Agreement for Medicine

Appendix 5: Record of Medicine Given

APPENDIX 1: BEING NOTIFIED OF A MEDICAL CONDITION

This is the process we follow when we are told that a pupil has a medical condition, or that a pupil's condition has changed.

1. **The school is told.** A parent or carer, the pupil, the placing local authority, a healthcare professional or the previous school tells us. The office records this and tells the Head Teacher the same day.
2. **The Head Teacher makes first contact.** The Head Teacher speaks to the family and the pupil to understand the condition and how it affects them, and puts sensible interim arrangements in place straight away if needed.
3. **Information is gathered.** The Head Teacher asks the family to complete the medical information form, and gets advice from the school nurse, GP, specialist or paediatrician where needed. If the pupil is new, this takes place before they start where possible.
4. **Decide if an IHP is needed.** The Head Teacher agrees with the family and a healthcare professional whether an IHP is needed. If they cannot agree, the Head Teacher decides. If no IHP is needed, the reason is recorded.
5. **Develop the IHP.** The Head Teacher meets the pupil, the family and healthcare professionals to write the plan (Appendix 3), identifies training needs and any staff who will give support, and checks insurance for any specialist procedure.
6. **Train and brief staff.** Staff who will support the pupil are trained and confirmed as competent, and staff who need to know are briefed. Supply and cover staff are included.
7. **Put the plan into practice.** The IHP is shared with the family and the staff involved, arrangements are in place within 2 weeks, or by the start of the relevant term for a new pupil, and the plan is linked to the pupil's EHC plan.
8. **Review.** The IHP is reviewed at least once a year, or sooner if the pupil's needs change. We go back to step 3 whenever new information comes in.

APPENDIX 2: SICK OR INFECTIOUS PUPILS

- Pupils who have an infectious disease should not come to school, and parents and carers should tell the school if their child has one.
- If a pupil becomes unwell during the day, for example with a temperature, sickness, diarrhoea or stomach pains, we will contact the parent or carer to collect their child. The pupil is kept with a trusted adult until they are collected, and is not left alone.
- Depending on the illness, staff may ask parents or carers to take their child to the doctor before they return.
- Staff will tell parents and carers if there is a risk to other pupils.
- Pupils with specific infectious diseases in the UKHSA exclusion table will not return until the recommended exclusion period has passed. Examples are below, and the Head Teacher checks the current UKHSA table whenever a case arises.

Illness	Usual guidance on returning to school
Chickenpox	Until all the spots have crusted over
Diarrhoea and vomiting	48 hours after the last episode
Flu	Until the pupil has recovered
Impetigo	Until the sores have crusted and healed, or 48 hours after starting antibiotics
Measles	4 days after the rash first appeared
Mumps	5 days after the swelling started
Scabies	Until after the first treatment
Scarlet fever	24 hours after starting antibiotics
Whooping cough	48 hours after starting antibiotics, or 21 days after the cough started if the pupil has not had antibiotics
Hand, foot and mouth disease, head lice, slapped cheek and conjunctivitis	Usually no exclusion is needed

We take the following steps to prevent the spread of infection:

- reducing sources of infection through good hygiene practices
- good handwashing, with soap and running water available to pupils and staff
- encouraging and supporting healthy eating
- following food hygiene requirements in the school kitchen areas and breakfast club
- regular cleaning, with extra cleaning when there is an outbreak (see the Health and Safety Policy)

- encouraging staff, parents, carers and pupils to take up immunisation as a protection against infection, while recognising each person's right to choose

APPENDIX 3: INDIVIDUAL HEALTHCARE PLAN

Complete this plan with the pupil, the parents or carers and a healthcare professional. Review it at least once a year, share a copy with the family, and keep it on the pupil's file. Link it to the pupil's EHC plan and Individual Support Plan.

INDIVIDUAL HEALTHCARE PLAN - to be completed with the pupil and family	
Pupil's name, date of birth, year group and date the plan was written:	
Home address and the emergency contact numbers of the parents or carers (at least two):	
Name, role and phone number of the GP, specialist or clinic, and any named nurse:	
Medical condition, triggers, signs and symptoms:	
The pupil's views: how the condition affects them and what helps:	
Daily care and treatment needed at school (including food and drink, rest, facilities, equipment and testing):	
Medicine: name, dose, method, time, side effects, storage, and who gives it:	
Does the pupil manage their own medicine? What help and monitoring are needed, and what happens if they refuse?	
Support for learning, social and emotional needs, exams and PE:	
Arrangements for school trips, visits and activities outside the school day (risk assessment completed?):	
What counts as an emergency for this pupil, and exactly what to do (who to call, what to give, when to dial 999):	
Staff who will give support, training completed, and the healthcare professional who confirmed their competence:	
Who needs to know about the condition, and any confidentiality requests:	
Cover arrangements for when the trained staff are not in school:	

Links to the EHC plan, Positive Behaviour Support Plan and any risk assessment:	
Review date and changes made at each review:	
Signed and dated: pupil, parent or carer, healthcare professional and Head Teacher:	

APPENDIX 4: PARENTAL AGREEMENT FOR MEDICINE

The school will not give your child medicine unless you complete and sign this form, and the Head Teacher has agreed that the school can do so.

PARENTAL AGREEMENT FOR THE SCHOOL TO GIVE MEDICINE	
Pupil's name and year group:	
Medical condition or illness:	
Name and strength of the medicine (as on the pharmacy label):	
Expiry date, and the date it was dispensed:	
Dose and how much to give:	
Method (for example tablet, liquid, inhaler, injection):	
Time or times to be given, and any special instructions or side effects:	
How long the medicine will be needed:	
Storage instructions, and whether the pupil may carry it:	
Name of the person to contact in an emergency, and a phone number:	
I understand that I must deliver the medicine personally to the school office in its original container, and that I must tell the school straight away in writing if the dose or instructions change or the medicine is stopped. Signed, name and date:	

APPENDIX 5: RECORD OF MEDICINE GIVEN

The member of staff giving the medicine completes this record each time, and the parent or carer is told the same day. Keep it with the pupil's IHP.

RECORD OF MEDICINE GIVEN TO A PUPIL	
Pupil's name and year group:	
Name and strength of the medicine, and the expiry date:	
Date and time the medicine was given:	
Dose given, and the time the previous dose was taken (checked with the parent or carer or the record):	
Any reaction, or if the pupil refused or the dose was missed (and what was done):	
Name and signature of the member of staff who gave the medicine, and the name of a second member of staff who witnessed it:	
Date and time the parent or carer was told, and by whom:	
Quantity of medicine remaining (for controlled drugs, the number held):	